Formulary Updates



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Following is the update to the Highmark Drug Formularies and pharmaceutical management procedures for April 2026. The formularies and pharmaceutical management procedures are updated after each Pharmacy and Therapeutics Committee meeting and the following changes reflect the decisions made in April by our Pharmacy and Therapeutics Committee. These updates are effective on the dates noted throughout this document.

Please reference the guide below to navigate this communication:

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All references to "Highmark" in this document are references to the Highmark company that is providing the member's health benefits or health benefit administration and/or to one or more of its affiliated Blue companies.

Availity is an independent company that contracts with Highmark to offer provider portal services.

As an added convenience, you can also search our drug formularies and view utilization management policies on the Provider Resource Center (PRC) (accessible via Availity Essentials® or our website). Once on the PRC, go to **Policies & Programs** > **Highmark Formulary** and then scroll down to find the formulary you're looking for.

Important Drug Safety Updates

FDA Is Requiring Warning about Vitamin B6 Deficiency and Associated Seizures for Drug Products Containing Carbidopa/Levodopa | FDA

On March 20, 2026, the FDA released a drug safety communication requiring all drug products containing carbidopa/levodopa add a warning to the prescribing information to state these medications can cause vitamin B6 deficiency and vitamin B6 deficiency-associated seizures. Healthcare professionals should evaluate baseline vitamin B6 levels prior to starting patients on carbidopa/levodopa therapies and periodically while on treatment, and supplement with vitamin B6 as necessary. Higher doses of carbidopa/levodopa may increase the risk of vitamin B6 deficiency. Seizures associated with the use of a product containing carbidopa and levodopa do not respond to traditional anti-seizure medications but resolve after vitamin B6 administration. Carbidopa/levodopa products are approved to treat symptoms of Parkinson's disease. Products may contain both carbidopa and levodopa (trade names: Crexont, Dhivy, Duopa, Rytary, Sinemet, and Sinemet CR); carbidopa/levodopa/entacapone (trade name: Stalevo); or foscarbidopa/foslevodopa (trade name: Vyalev), which is converted to active carbidopa/levodopa in the body. Products are available in multiple formulations and routes of administration. To help FDA monitor safety issues with medicines, report side effects involving drug products containing carbidopa/levodopa or other medicines to the FDA MedWatch program.

FDA Identifies Cases of Serious Liver Injury in Patients Taking Tavneos (avacopan) for Severe Active Anti-neutrophil Cytoplasmic Autoantibody (ANCA)-associated Vasculitis | FDA

On March 31, 2026, the FDA released a drug safety communication alerting about serious cases, including fatal cases, of drug-induced liver injury associated with Tavneos (avacopan) which is approved to treat severe active anti-neutrophil cytoplasmic autoantibody (ANCA)-associated vasculitis (granulomatosis with polyangiitis and microscopic polyangiitis). Some cases involved vanishing bile duct syndrome, which is characterized by progressive destruction and disappearance of the bile ducts in the liver. This condition can slow or stop the flow of bile and may lead to permanent liver damage. Although hepatotoxicity is a serious adverse reaction for Tavneos identified in premarket clinical trials and described in product labeling, vanishing bile duct syndrome and drug-induced liver injury cases with fatal outcomes represent new safety concerns. Patients should contact their health care professional immediately if they develop any signs or symptoms that may indicate liver injury, such as: feeling more tired than usual; nausea; vomiting; unusual itching; light-colored stools; yellowing of skin or eyes; dark urine; swelling in the stomach or abdomen; or pain in the right upper abdomen. Health care professionals should conduct liver panel tests every 2 weeks in the first month of treatment, monthly for the next 5 months, and then as clinically indicated. Discontinue Tavneos if alanine aminotransferase (ALT) or aspartate aminotransferase (AST) is >3 times the upper limit of normal (ULN) or alkaline phosphatase (ALP) is >2 times the ULN.

Highmark Formulary Update – April 2026

SECTION I. Highmark Commercial and Healthcare Reform Formularies

A. Changes to the Highmark Comprehensive Formulary and the Highmark Healthcare Reform Comprehensive Formulary

The Highmark Pharmacy and Therapeutics Committee has reviewed the medications listed in the tables below. Please note all medications added to the Comprehensive Closed/Incentive Formulary are also added to the Comprehensive Open Formulary. For your convenience, you can search the following formularies online:

- Highmark Comprehensive Formulary
- Highmark Healthcare Reform Comprehensive Formulary

Highmark is happy to inform you that Table 1 includes products that have been added to the formulary. Adding products to the formulary may mean lower copays or coinsurance rates for members. By adding products to the formulary, Highmark hopes to promote adherence to medication protocols and improve the overall health of our members.

Table 1. Products Added

All products added to the formulary.

Brand Name	Generic Name	Comments
Prezcobix PED	darunavir and cobicistat	Treatment of HIV-1 in treatment-naïve and treatment-experienced adults and pediatric patients 3 years of age and older

Coverage may be contingent upon plan benefits.

Table 2. Products Not Added**

Brand Name	Generic Name	Preferred Alternatives
Adquey	difamilast	tacrolimus ointment
Bimatoprost PF	Bimatoprost PF	Prescriber discretion
Bysanti	milsaperidone	quetiapine tablets, aripiprazole tablets
Desmoda	desmopressin acetate	desmopressin acetate tablet
Filkri	filgrastim-laha	Zarxio Nivestym
Nereus	tradipitant	scopolamine meclizine HCl tablet 12.5 mg meclizine HCl tablet 25 mg
Ontralfy	tizanidine oral solution	tizanidine tablets
Pokonza oral solution	potassium chloride	potassium chloride 20 mEq tablet extended release potassium chloride capsules extended release

Brand Name	Generic Name	Preferred Alternatives
Yuvezzi	carbachol and brimonidine tartrate ophthalmic solution	Prescriber discretion
Yuviwel	navepegritide	Prescriber discretion
Zycubo	copper histidinate	Prescriber discretion

Coverage may be contingent upon plan benefits.
Physicians may request coverage of these products using the Prescription Drug Medication Request Form. To access this form for your region, go to the Provider Resource Center and choose your region from the top right. Select **Resources & Education > Forms > Pharmacy Prior Authorization Forms and then scroll down to the **Prescription Drug Medication Request Form**.

Table 3. Additions to the Specialty Tier Copay Option

Note: The specialty tier does not apply to Highmark Delaware Healthcare Reform members; see Highmark Delaware's online Provider Resource Center and access the **Pharmacy Program/Formularies** link for details on the formularies and formulary options that apply to Highmark Delaware Healthcare Reform members. Once on the page, click on **Heathcare Reform**, which is under the "Line of Business" header.

Brand Name	Generic Name
Adquey	difamilast
Bysanti	milsaperidone
Desmoda	desmopressin acetate
Filkri	filgrastim-laha
Ontralfy	tizanidine oral solution
Pokonza oral solution	potassium chloride
Yuviwel	navepegritide
Zycubo	copper histidinate

Table 4. Products to Be Removed or Shifted to Higher Tier – Effective July 2026

Brand name	Generic Name	Preferred Alternatives
Commercial Comprehensive products		
Edurant 25 mg tablet	rilpivirine hcl	rilpivirine 25 mg tablet
Commercial & Healthcare Reform Comprehensive products		
Endometrin 100 mg vag insert	progesterone, micronized	progesterone 100 mg vag insert
Farxiga 10 mg tablet	dapagliflozin propanediol	dapagliflozin 10 mg tablet
Farxiga 5 mg tablet	dapagliflozin propanediol	dapagliflozin 5 mg tablet
Venofer 100 mg/5 mL vial	iron sucrose complex	iron sucrose 100 mg/5 mL vial
Venofer 200 mg/10 mL vial	iron sucrose complex	iron sucrose 200 mg/10 mL vial
Venofer 50 mg/2.5 mL vial	iron sucrose complex	iron sucrose 50 mg/2.5 mL vial
Xigduo XR 10 mg-1,000 mg tab	dapagliflozin propanediol/metformin	dapagliflozin-metformin ER 10-1000

Brand name	Generic Name	Preferred Alternatives
Xigduo XR 10 mg-500 mg tablet	dapagliflozin propanediol/metformin	dapagliflozin- metformin ER 10-500
Xigduo XR 2.5 mg-1,000 mg tab	dapagliflozin propanediol/metformin	generic anticipated
Xigduo XR 5 mg-1,000 mg tablet	dapagliflozin propanediol/metformin	dapagliflozin- metformin ER 5-1000
Xigduo XR 5 mg-500 mg tablet	dapagliflozin propanediol/metformin	dapagliflozin- metformin ER 5-500

B. Changes to the Highmark Healthcare Reform Essential Formulary

The Essential Formulary is a closed formulary for select Healthcare Reform (HCR) Individual plans. A list of drugs included on the Essential Formulary, listed by therapeutic class, is available here.

Table 1. Formulary Updates

Brand Name	Generic Name	Tier	Comments/Preferred Alternatives
Items listed below were added to the formulary			
Prezcobix PED	darunavir and cobicistat	3	Treatment of HIV-1 in treatment-naïve and treatment-experienced adults and pediatric patients 3 years of age and older
Items listed below were not added to the formulary			
Adquey	difamilast	NF	tacrolimus ointment pimecrolimus
Bimatoprost PF	Bimatoprost PF	NF	Prescriber discretion
Bysanti	milsaperidone	NF	quetiapine tablets aripiprazole tablets
Desmoda	desmopressin acetate	NF	desmopressin acetate tablet
Filkri	filgrastim-laha	NF	Zarxio Nivestym
Nereus	tradipitant	NF	scopolamine meclizine HCl tablet 12.5 mg meclizine HCl tablet 25 mg
Ontralfy	tizanidine oral solution	NF	tizanidine tablets
Pokonza oral solution	potassium chloride	NF	potassium chloride 20 mEq tablet extended release potassium chloride capsules extended release
Yuvezzi	carbachol and brimonidine tartrate ophthalmic solution	NF	Prescriber discretion
Yuviwel	navepegritide	NF	Prescriber discretion
Zycubo	copper histidinate	NF	Prescriber discretion

Formulary options: **Tier 1:** Generic drugs; **Tier 2:** Generic and Brand drugs; **Tier 3:** Generic and Brand drugs; **Tier 4:** Generic and Brand drugs; **Non-formulary (NF)**.
*Effective date to be determined.

Table 2. Products to Be Removed or Shifted to Higher Tier – Effective July 2026

Brand Name	Generic Name	Preferred Alternatives
All Healthcare Reform Essential Products		
Auryxia 210 mg tablet	ferric citrate	ferric citrate 210 mg tablet
Briviact 10 mg tablet	brivaracetam	brivaracetam 10 mg tablet
Briviact 100 mg tablet	brivaracetam	brivaracetam 100 mg tablet
Briviact 25 mg tablet	brivaracetam	brivaracetam 25 mg tablet
Briviact 50 mg tablet	brivaracetam	brivaracetam 50 mg tablet
Briviact 75 mg tablet	brivaracetam	brivaracetam 75 mg tablet
Edurant 25 mg tablet	rilpivirine hcl	rilpivirine 25 mg tablet
Endometrin 100 mg vag insert	progesterone, micronized	progesterone 100 mg vag insert
Farxiga 10 mg tablet	dapagliflozin propanediol	dapagliflozin 10 mg tablet
Farxiga 5 mg tablet	dapagliflozin propanediol	dapagliflozin 5 mg tablet
Gleostine 10 mg capsule	lomustine	lomustine 10 mg capsule
Gleostine 100 mg capsule	lomustine	lomustine 100 mg capsule
Gleostine 40 mg capsule	lomustine	lomustine 40 mg capsule
Ofev 100 mg capsule	nintedanib esylate	nintedanib esylate 100 mg cap
Ofev 150 mg capsule	nintedanib esylate	nintedanib esylate 150 mg cap
Pomalyst 1 mg capsule	pomalidomide	pomalidomide 1 mg capsule
Pomalyst 2 mg capsule	pomalidomide	pomalidomide 2 mg capsule
Pomalyst 3 mg capsule	pomalidomide	pomalidomide 3 mg capsule
Pomalyst 4 mg capsule	pomalidomide	pomalidomide 4 mg capsule
Savella 100 mg tablet	milnacipran hcl	milnacipran HCl 100 mg tablet
Savella 12.5 mg tablet	milnacipran hcl	milnacipran HCl 12.5 mg tablet
Savella 25 mg tablet	milnacipran hcl	milnacipran HCl 25 mg tablet
Savella 50 mg tablet	milnacipran hcl	milnacipran HCl 50 mg tablet
Savella titration pack	milnacipran hcl	milnacipran HCl titration pack
Xigduo XR% 10 mg-1,000 mg tab	dapagliflozin propanediol/metformin	dapagliflozin-metformin ER 10-1000
Xigduo XR 10 mg-500 mg tablet	dapagliflozin propanediol/metformin	dapagliflozin- metformin ER 10-500
Xigduo XR 2.5 mg-1,000 mg tab	dapagliflozin propanediol/metformin	generic anticipated
Xigduo XR 5 mg-1,000 mg tablet	dapagliflozin propanediol/metformin	dapagliflozin- metformin ER 5-1000
Xigduo XR 5 mg-500 mg tablet	dapagliflozin propanediol/metformin	dapagliflozin- metformin ER 5-500

C. Changes to the Highmark Core Formulary

The Core Formulary is a closed formulary for select Commercial Individual plans. A list of drugs included on the Core Formulary, listed by therapeutic class, is available here.

Table 1. Formulary Updates

Brand Name	Generic Name	Tier	Comments/Preferred Alternatives
Items listed below were added to the formulary			
Prezcobix PED	darunavir and cobicistat	3	Treatment of HIV-1 in treatment-naïve and treatment-experienced adults and pediatric patients 3 years of age and older
Items listed below were not added to the formulary			
Adquey	difamilast	NF	tacrolimus ointment pimecrolimus
Bimatoprost PF	Bimatoprost PF	NF	Prescriber discretion
Bysanti	milsaperidone	NF	quetiapine tablets aripiprazole tablets
Desmoda	desmopressin acetate	NF	desmopressin acetate tablet
Filkri	filgrastim-laha	NF	Nivestym
Nereus	tradipitant	NF	scopolamine meclizine HCl tablet 12.5 mg meclizine HCl tablet 25 mg
Ontralfy	tizanidine oral solution	NF	tizanidine tablets
Pokonza oral solution	potassium chloride	NF	potassium chloride 20 mEq tablet extended release potassium chloride capsules extended release
Yuvezzi	carbachol and brimonidine tartrate ophthalmic solution	NF	Prescriber discretion
Yuviwel	navepegritide	NF	Prescriber discretion
Zycubo	copper histidinate	NF	Prescriber discretion

Formulary options: **Tier 1:** Generic drugs; **Tier 2:** Generic and Brand drugs; **Tier 3:** Generic and Brand drugs; **Tier 4:** Generic and Brand drugs; **Non-formulary (NF)**.

Table 2. Products to Be Removed or Shifted to Higher Tier – Effective July 2026

Brand Name	Generic Name	Preferred Alternatives
All Core Products		
Auryxia 210 mg tablet	ferric citrate	ferric citrate 210 mg tablet
Edurant 25 mg tablet	rilpivirine hcl	rilpivirine 25 mg tablet
Endometrin 100 mg vag insert	progesterone micronized	progesterone 100 mg vag insert
Farxiga 10 mg tablet	dapagliflozin propanediol	dapagliflozin 10 mg tablet
Farxiga 5 mg tablet	dapagliflozin propanediol	dapagliflozin 5 mg tablet

Gleostine 10 mg capsule	lomustine	lomustine 10 mg capsule
Gleostine 100 mg capsule	lomustine	lomustine 100 mg capsule
Gleostine 40 mg capsule	lomustine	lomustine 40 mg capsule
Lortab 10 mg-300 mg/15 ml elixir	hydrocodone/acetaminophen	hydrocodone-acetaminophen 10-300/15
Ofev 100 mg capsule	nintedanib esylate	nintedanib esylate 100 mg cap
Ofev 150 mg capsule	nintedanib esylate	nintedanib esylate 150 mg cap
Xigduo XR 10 mg-1,000 mg tab	dapagliflozin propanediol/metformin	dapagliflozin-metformin ER 10-1000
Xigduo XR 10 mg-500 mg tablet	dapagliflozin propanediol/metformin	dapagliflozin- metformin ER 10-500
Xigduo XR 2.5 mg-1,000 mg tab	dapagliflozin propanediol/metformin	generic anticipated
Xigduo XR 5 mg-1,000 mg tablet	dapagliflozin propanediol/metformin	dapagliflozin- metformin ER 5-1000
Xigduo XR 5 mg-500 mg tablet	dapagliflozin propanediol/metformin	dapagliflozin- metformin ER 5-500

D. Changes to the Highmark National Select Formulary

The National Select Formulary is an incentive formulary with a non-formulary drug list to manage products in therapeutic categories for which preferred alternatives are available. The National Select Formulary is available for select Commercial self-funded (ASO) plans. A list of drugs included on the National Select Formulary, listed by therapeutic class, is available here.

Table 1. Formulary Updates

Brand Name	Generic Name	Tier	Comments/Preferred Alternatives
Items listed below were added to the formulary (Non-Preferred)			
Adquey	difamilast	3*	tacrolimus ointment (gram) Eucrisa
Bimatoprost PF	Bimatoprost PF	3	Prescriber discretion
Bysanti	milsaperidone	3*	quetiapine tablets aripiprazole tablets
Filkri	filgrastim-laha	3*	Nivestym
Nereus	tradipitant	3*	scopolamine meclizine hcl tablet 12.5 mg meclizine hcl tablet 25 mg
Prezcobix PED	darunavir and cobicistat	3*	Prescriber discretion
Yuviwel	navepegritide	3*	Prescriber discretion
Items listed below were not added to the formulary			
Ontralfy	tizanidine oral solution	NF	tizanidine tablets
Pokonza oral solution	potassium chloride	NF	Potassium chloride 20 mEq tablet extended release, potassium chloride capsules extended release

Brand Name	Generic Name	Tier	Comments/Preferred Alternatives
Desmoda	desmopressin acetate	NF	desmopressin acetate tablet
Yuvezzi	carbachol and brimonidine tartrate ophthalmic solution	NF	Prescriber discretion
Zycubo	copper histidinate	NF	Prescriber discretion

Formulary options: **Tier 1:** Generic drugs; **Tier 2:** Preferred Brand drugs; **Tier 3:** Non-Preferred Brand drugs; **Non-formulary (NF)**.
*Effective date and final formulary position to be determined.

Table 2. Additions to the Specialty Tier Copay Option

Brand Name	Generic Name
Adquey	difamilast
Bysanti	milsaperidone
Desmoda	desmopressin acetate
Filkri	filgrastim-laha
Ontralfy	tizanidine oral solution
Pokonza oral solution	potassium chloride
Yuviwel	navepegritide
Zycubo	copper histidinate

Table 3. Products to Be Removed or Shifted to Higher Tier – Effective July 2026

Brand Name	Generic Name	Preferred Alternatives
All National Select Products		
Entresto 24 mg-26 mg tablet	sacubitril/valsartan	sacubitril-valsartan 24-26 mg
Entresto 49 mg-51 mg tablet	sacubitril/valsartan	sacubitril-valsartan 49-51 mg
Entresto 97 mg-103 mg tablet	sacubitril/valsartan	sacubitril-valsartan 97-103 mg
Farxiga 10 mg tablet	dapagliflozin propanediol	dapagliflozin 10 mg tablet
Farxiga 5 mg tablet	dapagliflozin propanediol	dapagliflozin 5 mg tablet
Fluvastatin er 80 mg tablet	fluvastatin sodium	atorvastatin 20 mg tablet rosuvastatin calcium 5 mg tab
Fluvastatin sodium 20 mg cap	fluvastatin sodium	lovastatin 20 mg tablet pravastatin sodium 20 mg tab
Fluvastatin sodium 40 mg cap	fluvastatin sodium	lovastatin 20 mg tablet pravastatin sodium 20 mg tab
Lescol xl 80 mg tablet	fluvastatin sodium	atorvastatin 20 mg tablet

		rosuvastatin calcium 5 mg tab
Livdelzi 10 mg capsule	seladelpar lysine	Iqirvo 80 mg tablet
Mavenclad 10 mg x 10 tablet pk	cladribine	cladribine 10 mg x 10 tab pk
Mavenclad 10 mg x 4 tablet pk	cladribine	cladribine 10 mg x 4 tablet pk
Mavenclad 10 mg x 5 tablet pk	cladribine	cladribine 10 mg x 5 tablet pk
Mavenclad 10 mg x 6 tablet pk	cladribine	cladribine 10 mg x 6 tablet pk
Mavenclad 10 mg x 7 tablet pk	cladribine	cladribine 10 mg x 7 tablet pk
Mavenclad 10 mg x 8 tablet pk	cladribine	cladribine 10 mg x 8 tablet pk
Mavenclad 10 mg x 9 tablet pk	cladribine	cladribine 10 mg x 9 tablet pk
Mydayis ER 12.5 mg capsule	dextroamphetamine/amphetamine	dextroamph-amphet ER 12.5mg cp
Mydayis ER 25 mg capsule	dextroamphetamine/amphetamine	dextroamph-amphet ER 25 mg cap
Mydayis ER 37.5 mg capsule	dextroamphetamine/amphetamine	dextroamph-amphet ER 37.5mg cp
Mydayis ER 50 mg capsule	dextroamphetamine/amphetamine	dextroamph-amphet ER 50 mg cap
Norgesic 25-385-30 mg tablet	orphenadrine/aspirin/caffeine	orphenadrine ER 100 mg tablet
Norgesic Forte 50-770-60 mg tb	orphenadrine/aspirin/caffeine	orphenadrine ER 100 mg tablet
Orphenad-Asa-Caff 50-770-60 mg	orphenadrine/aspirin/caffeine	orphenadrine ER 100 mg tablet
Orphenadrn-Asa-Caf 25-385-30mg	orphenadrine/aspirin/caffeine	orphenadrine ER 100 mg tablet
Orphengesic Forte 50-770-60 mg	orphenadrine/aspirin/caffeine	orphenadrine ER 100 mg tablet
Osenvelt 120 mg/1.7 ml vial	denosumab-bmwo	Bilprevda 120 mg/1.7 mL vial, Xtrenbo 120 mg/1.7 mL vial
Protriptyline HCl 10 mg tablet	protriptyline hcl	nortriptyline HCl 10 mg cap amitriptyline HCl 10 mg tab
Protriptyline HCl 5 mg tablet	protriptyline hcl	nortriptyline HCl 10 mg cap amitriptyline HCl 10 mg tab
Xgeva 120 mg/1.7 ml vial	denosumab	Bilprevda 120 mg/1.7 mL vial Xtrenbo 120 mg/1.7 mL vial
Xigduo XR 10 mg-1,000 mg tab	dapagliflozin propanediol/metformin	dapagliflozin- metformin ER 10-1000

Xigduo XR 10 mg-500 mg tablet	dapagliflozin propanediol/metformin	dapagliflozin- metformin ER 10-500
Xigduo XR 2.5 mg-1,000 mg tab	dapagliflozin propanediol/metformin	generic anticipated
Xigduo XR 5 mg-1,000 mg tablet	dapagliflozin propanediol/metformin	dapagliflozin- metformin ER 5-1000
Xigduo XR 5 mg-500 mg tablet	dapagliflozin propanediol/metformin	dapagliflozin- metformin ER 5-500

E. Updates to the Pharmacy Utilization Management Programs

1. Prior Authorization Program

Policy Name*	Policy Effective Date**	Updates and/or Approval Criteria
Abiraterone Products – Commercial and Healthcare Reform	04/28/2026	Policy revised to add abiraterone acetate, micronized as a target requiring age and criteria based on FDA-approved indication; use in combination with a gonadotropin releasing hormone analog or the member has had a bilateral orchiectomy; and the member has experienced therapeutic failure or intolerance to generic abiraterone acetate 250 mg tablets or Abirtega (abiraterone acetate). Policy revised for brand Yonsa (abiraterone acetate, micronized) to require therapeutic failure or intolerance to generic abiraterone acetate, micronized.
Achondroplasia – Commercial and Healthcare Reform	04/28/2026	Policy revised to add Yuviwel (navepegritide) as a target requiring FDA-approved age, genetic confirmation of achondroplasia, baseline annualized growth velocity (AGV), and open epiphyses. Reauthorization requiring an increase in AGV compared to baseline, open epiphysis, and for both Yuviwel and Voxzogo (vosoritide), age less than 18 years.
Acthar Gel Vial (repository corticotropin) – Commercial and Healthcare Reform – New York, Pennsylvania, West Virginia, and Delaware ASO	Effective upon completion of internal review and implementation	Policy revised to remove targets Acthar Gel Selfject (repository corticotropin) and Purified Cortrophin (repository corticotropin) and move to J-1459. Request for Acthar Gel Vial (repository corticotropin) for members 2 years of age or older will also move to J-1459. Request for Delaware members with commercial

Policy Name*	Policy Effective Date**	Updates and/or Approval Criteria
		full insured or healthcare reform removed from policy.
Adquey (difamilast) Vtama (tapinarof) and Zoryve (roflumilast) – Commercial and Healthcare Reform	Effective upon completion of internal review and implementation	Policy revised to add Adquey (difamilast) for atopic dermatitis requiring age, FDA-approved diagnosis, and failure/intolerance/contraindication to generic topical tacrolimus or pimecrolimus. Reauthorization of positive clinical response to therapy.
ALK-Targeting Kinase Inhibitors – Commercial and Healthcare Reform	04/28/2026	Policy revised for Ensacove (ensartinib) to require that disease is ALK-positive, as detected by an FDA-approved test.
Anti-EGFR and HER2 Kinase Inhibitors – Commercial and Healthcare Reform	04/28/2026	Policy revised for Hernexeos (zongertinib) to remove requirement of prior systemic therapy based on updated FDA-approved indication for non-small cell lung cancer.
Antifibrotic Pulmonary Medications – Commercial and Healthcare Reform	04/28/2026	Policy revised for Jascayd (nerandomilast) in the treatment of idiopathic pulmonary fibrosis (IPF) to change the criterion requiring a baseline forced vital capacity (FVC) from at least 50% to at least 45%.
Anti-Obesity (Enhanced) – Commercial and Healthcare Reform	04/03/2026	Policy revised to add Wegovy HD (semaglutide) as a target for weight loss. Requirements for maintenance dosing updated to include Wegovy HD 7.2 mg, as appropriate based age and indication per FDA-approved maintenance dosing recommendations. For adolescents, policy revised for all targets to require FDA-approved age.
Anti-Obesity (Enhanced) – Commercial and Healthcare Reform (ad-hoc)	02/19/2026	Criteria revised for all targets for all indications to define baseline BMI as measured within 6 months of initiating the requested medication (and/or 6 months of initiating a GLP-1 medication, if the request is for a GLP-1).
Anti-Obesity (Enhanced) – Commercial and Healthcare Reform (ad-hoc)	Effective upon completion of internal review and implementation	Policy revised for Zepbound (tirzepatide) single-dose injection to require contraindication to Zepbound Kwikpen.
Anti-Obesity (Standard) – Commercial and Healthcare Reform	04/03/2026	Policy revised to add Wegovy HD (semaglutide) as a target. Requirements for maintenance dosing updated to include Wegovy HD 7.2 mg, as appropriate based age per FDA-approved maintenance dosing recommendations.

Policy Name*	Policy Effective Date**	Updates and/or Approval Criteria
		For adolescents, policy revised for all targets to require FDA-approved age.
Anti-Obesity (Standard) – Commercial and Healthcare Reform (ad-hoc)	Effective upon completion of internal review and implementation	Policy revised for Zepbound (tirzepatide) single-dose injection to require contraindication to Zepbound Kwikpen.
BRAF Mutation-Targeting & MEK Kinase Inhibitors – Commercial and Healthcare Reform	04/28/2026	Policy revised for Braftovi (encorafenib) to require diagnosis based on updated indication for metastatic colorectal cancer used in combination with cetuximab and fluorouracil-based chemotherapy.
Branded Hydroxyurea Products – Commercial and Healthcare Reform	04/28/2026	Policy revised for Xromi (hydroxyurea) to allow trial/failure of generic hydroxyurea or inability to swallow tablets.
Chronic Inflammatory Diseases – Commercial and Healthcare Reform	04/28/2026	Policy revised for Sotyktu (deucravacitinib) to add new indication of psoriatic arthritis (PsA) requiring the member is 18 years or older, prescribed by or in consultation with a specialist, spinal or axial PsA with trial of at least one nonsteroidal anti-inflammatory drug (NSAID), or PsA without spinal disease with trial of at least one non-biologic disease modifying antirheumatic drug (DMARD), or enthesitis and/or dactylitis associated with PsA with trial of at least 1 NSAID or local glucocorticoid injection, and step through 2 preferred step 1 or 2a agents for PsA. Policy revised to include Avtozma SC (tocilizumab-anoh) as a non-preferred biosimilar and for Cosentyx (secukinumab) to update hidradenitis suppurativa age restriction to 12 years of age and older.
Chronic Inflammatory Diseases – Commercial and Healthcare Reform	04/08/2026	If a clinical exception is determined medically necessary for Simlandi per policy J-1492, the member must experience therapeutic failure or intolerance to Hadlima. If a clinical exception is determined medically necessary for Yesintek SC per policy J-1492, the member must experience therapeutic failure or intolerance to Steqeyma SC.
Chronic Inflammatory Diseases – Commercial	04/28/2026	Policy revised for Sotyktu (deucravacitinib) to add new indication of

Policy Name*	Policy Effective Date**	Updates and/or Approval Criteria
National Select Formulary		psoriatic arthritis (PsA) requiring the member is 18 years or older, prescribed by or in consultation with a specialist, spinal or axial PsA with trial of at least one nonsteroidal anti-inflammatory drug (NSAID), or PsA without spinal disease with trial of at least one non-biologic disease modifying antirheumatic drug (DMARD), or enthesitis and/or dactylitis associated with PsA with trial of at least 1 NSAID or local glucocorticoid injection. Policy revised to include Avtozma SC (tocilizumab-anoh) as a non-preferred biosimilar and for Cosentyx (secukinumab) to update hidradenitis suppurativa age restriction to 12 years of age and older.
Chronic Inflammatory Diseases – Commercial National Select Formulary	04/08/2026	If a clinical exception is determined medically necessary for Simlandi per policy J-1492, the member must experience therapeutic failure or intolerance to Hadlima. If a clinical exception is determined medically necessary for Yesintek SC per policy J-1492, the member must experience therapeutic failure or intolerance to Steqeyma SC.
Denosumab Products for Bone Disease and Evenity (romosozumab-aqqg) – Commercial and Healthcare Reform	04/28/2026	Policy revised to update definitions of high and very high fracture risk to match American Association of Clinical Endocrinologists/American College of Endocrinology. For denosumab products, patients at very high fracture risk do not have to try/fail bisphosphonates.
Desmoda (desmopressin acetate) – Commercial and Healthcare Reform	04/28/2026	New policy created for Desmoda (desmopressin acetate), initial authorization criteria requiring prescribed by or in consultation with an endocrinologist or a provider who specializes in central diabetes insipidus, diagnosis, the member meets all of the following creatine clearance is ≥ 50 mL/min and serum sodium concentration is ≥ 135 mEq/L, the member has experienced therapeutic failure, contraindication, or intolerance to generic

Policy Name*	Policy Effective Date**	Updates and/or Approval Criteria
		desmopressin acetate tablets, and has an inability to swallow tablets. For reauthorization, the member has experienced positive clinical response to therapy, the member meets all of the following creatine clearance is ≥ 50 mL/min and serum sodium concentration is ≥ 135 mEq/L, the member has experienced therapeutic failure, contraindication, or intolerance to generic desmopressin acetate tablets, and has an inability to swallow tablets. Quantity level limits were added requiring member's age, and for quantities more than 1 bottle for month, Desmoda may be authorized in the following quantities; if ≤ 12 years of age: 4 bottles (580 mL) per month or if > 12 years of age: 5 bottles (725 mL) per month. Initial authorization duration of 6 months, and reauthorization duration of 12 months.
Dupixent (dupilumab) – Commercial and Healthcare Reform	4/28/206	Policy revised for Dupixent (dupilumab) for new indication of allergic fungal rhinosinusitis (AFRS) to require age, weight >/= 15 kg, FDA-approved diagnosis, and diagnosis confirmed by two of the following: Type 1 hypersensitivity to fungus confirmed by history, in vitro IgE immunoassays, or skin testing; positive fungal stain of sinus contents removed during surgery; presence of eosinophilic mucin without invasion into the sinus tissue; presence of chronic rhinosinusitis with nasal polyposis (NP), or characteristic CT signs. Reauthorization requires decrease in nasal polyp score or nasal congestion/obstruction severity score.
Evrysdi (risdiplam) – Commercial and Healthcare Reform	04/28/2026	Policy revised to require that the member has molecular genetic testing results documenting 0 copies of SMN1, molecular genetic testing results documenting 2, 3, or 4 copies of SMN2, and chart notes documenting baseline motor function test results. Initial and reauthorization revised to require the member does not require

Policy Name*	Policy Effective Date**	Updates and/or Approval Criteria
		invasive ventilation or tracheostomy, prescriber confirmation that member has not received gene therapy and if the member has received gene therapy, there is clinical documentation of decline in clinical status as demonstrated by decline in functional score(s).
Familial Chylomicronemia Syndrome (FCS) Medications – Commercial and Healthcare Reform	04/02/2026	Policy updated to remove step in initial and reauthorization criteria requiring, if the request is for Tryngolza (olezarsen) the member has experienced therapeutic failure or intolerance to Redemplo (plozasiran).
HIF PH Inhibitors – Commercial and Healthcare Reform	04/28/2026	Policy revised for Jesduvroq (daprodustat) and Vafseo (vadadustat) to remove step through intravenous iron therapy.
Homozygous Familial Hypercholesterolemia – Commercial and Healthcare Reform	04/28/2026	Juxtapid (lomitapide) initial authorization criteria was revised to account for new age expansion of 2 years of age or older. Criteria now requires an LDL > 115 mg/dL for members 17 years of age or younger or LDL > 70 mg/dL if 18 years of age or older or LDL > 55 mg/dL if 18 years of age and older and atherosclerotic cardiovascular disease (ASCVD) or major ASCVD risk factors. Additionally, criteria now requires if the member is 10 years of age or older, the member has experienced therapeutic failure, contraindication, or intolerance to ezetimibe and Repatha.
Human Growth Hormone – Commercial and Healthcare Reform	04/28/2026	Policy revised for Sogroya (somapacitan-beco) to allow approval for expanded indication of pediatric patients at least 2.5 years of age with short stature who were small for gestational age or have growth failure due to Noonan syndrome, based on previously approved criteria for these indications. For all targets for initial authorization of Noonan syndrome, if the request is for a non-preferred product, the member has experienced failure or intolerance to all preferred products.
Human Growth Hormone – Commercial	04/28/2026	Policy revised for Sogroya (somapacitan-beco) to allow approval for expanded indication of pediatric patients at least 2.5

Policy Name*	Policy Effective Date**	Updates and/or Approval Criteria
and Healthcare Reform – Delaware		years of age with short stature who were small for gestational age, idiopathic short stature, or have growth failure due to Noonan syndrome, based on previously approved criteria for these indications. For all targets for initial authorization of Noonan syndrome, if the request is for a non-preferred product, the member has experienced failure or intolerance to all preferred products.
Imcivree (setmelanotide) – Commercial and Healthcare Reform	04/28/2026	Policy revised for Imcivree (setmelanotide) to require FDA-approved age and diagnosis of hypothalamic obesity, and baseline height, weight, and BMI. For patients 4 to 19 years of age, baseline weight must be ≥ 95th percentile, and for patients at least 20 years of age, baseline BMI must be ≥ 30 kg/m^2. Reauthorization added for continuation that member has experienced at least 5% weight loss from baseline and for maintenance member has maintained weight loss from baseline.
Isturisa (osilodrostat) – Commercial and Healthcare Reform (ad-hoc)	02/19/2026	Policy revised for Isturisa (osilodrostat) to require endocrinologist prescribing for initial authorization. For initial and reauthorization, requiring documentation that the members has failed, or is not a candidate for, surgery.
Journavx (suzetrigine) – Commercial and Healthcare Reform	04/28/2026	Policy revised to include revised indication: moderate-to-severe acute pain, including postoperative pain, in adults. No changes to criteria necessary.
Liquid Formulation of Amlodipine Products – Commercial and Healthcare Reform	04/28/2026	Policy revised to make Sdamlo (amlodipine) oral solution a miscellaneous specialty drug oral benefit and if the request is for Sdamlo, the member has experienced therapeutic failure or intolerance to plan-preferred Katerzia and Norliqva.
Lumryz (sodium oxybate), Xyrem (sodium oxybate) and Xywav (calcium, magnesium, potassium, and sodium oxybates) –	04/28/2026	Policy revised to add step through generic sodium oxybate for members 18 years of age or older requesting brand Xyrem (sodium oxybate), Lumryz (sodium oxybate), or Xywav (calcium, magnesium,

Policy Name*	Policy Effective Date**	Updates and/or Approval Criteria
Commercial and Healthcare Reform		potassium, and sodium oxybates) for initial and reauthorization.
Metabolic Dysfunction – Associated Steatohepatitis (MASH) – Commercial and Healthcare Reform	Effective upon completion of internal review and implementation	Policy revised to allow ELF blood test, within 6 months of the initial medication request, as an approvable non-invasive test.
Miscellaneous Immunomodulators – Commercial and Healthcare Reform	04/28/2026	Policy revised to add generic pomalidomide as a target.
Miscellaneous Immunomodulators – Commercial and Healthcare Reform	Effective upon completion of internal review and implementation	Policy revised for brand Pomalyst (pomalidomide) to require therapeutic failure or intolerance to generic pomalidomide for initial and reauthorization.
Nereus (tradipitant) – Commercial and Healthcare Reform	Effective upon completion of internal review and implementation	New policy for Nereus (tradipitant) requiring FDA approved diagnosis and age and therapeutic failure, contraindication, or intolerance to generic scopolamine patch and one generic first generation antihistamine (for example, promethazine, meclizine, dimenhydrinate). Quantity limit requiring documentation of event expected to cause vomiting by motion for more than 2 days, documentation of the number of days that member will be exposed to an event expected to cause vomiting by motion, and that the member has not received more than 90 doses per lifetime.
Non-preferred Oral Potassium Chloride Products – Commercial and Healthcare Reform	04/28/2026	Policy revised to add Pokonza oral solution (potassium chloride) as a target mirroring criteria for Pokonza pouches for oral solution (potassium chloride).
Noxafil (posaconazole) – Commercial and Healthcare Reform	04/28/2026	Policy updated to align with FDA-approved indication. Policy also updated if the request is for Noxafil Powder Mix for delayed-release oral suspension (posaconazole), the member has a diagnosis of invasive aspergillosis infection, is 2 years of age and older, weighs > 40 kg, the member has experienced therapeutic failure, contraindication, or intolerance to

Policy Name*	Policy Effective Date**	Updates and/or Approval Criteria
		voriconazole, and the member is unable to swallow tablets or capsules.
Palynziq (pegvaliase-pqpz) – Commercial and Healthcare Reform	04/28/2026	Policy revised for Palynziq (pegvaliase-pqpz) to revise required age to 12 years of age or older. Reauthorization criteria revised to require the member has a blood phenylalanine (Phe) level ≤ 600 µM/L or if > 600µM/L, the member meets all of the following: the prescriber attests that the member is still undergoing titration to an effective maintenance dose and the prescriber attest that the drug will be discontinued if the members blood Phe concentration is still > 600µM/L after 16 weeks of Palynziq 60 mg daily.
Parathyroid Hormone Analogs – Commercial and Healthcare Reform	04/28/2026	Policy revised to update definitions of high and very high fracture risk to match American Association of Clinical Endocrinologists/American College of Endocrinology.
PCSK9 Inhibitors – Commercial and Healthcare Reform	Effective upon completion of internal review and implementation	Policy revised to lower LDL-C from ≥ 90 to ≥70 in initial authorization for primary prevention indication. Changed verbiage from Primary Hyperlipidemia to Hypercholesterolemia to align with package insert and also removed the member meets one (1) of the following, the member has a coronary artery calcium of calcification score (CAC) score ≥ 1,000 Agatston units and the member has a baseline untreated LDL ≥ 190 from initial authorization.
Presbyopia Products – Commercial and Healthcare Reform	04/28/2026	Policy revised to add Yuvezzi (carbachol and brimonidine tartrate) requiring age of 45 years or older, diagnosis of presbyopia, therapeutic failure of one or contraindication to all: eyeglasses or contact lenses, and therapeutic failure, intolerance, or contraindication to plan-preferred generic pilocarpine 1.25% ophthalmic solution. Reauthorization requiring positive clinical response to therapy and therapeutic failure, intolerance, or contraindication to plan-preferred generic pilocarpine 1.25% ophthalmic solution. Reauthorization for

Policy Name*	Policy Effective Date**	Updates and/or Approval Criteria
		Qlosi (pilocarpine hydrochloride), Vizz (aceclidine), and Yuvezzi (carbachol and brimonidine tartrate) updated to require therapeutic failure, intolerance, or contraindication to plan-preferred generic pilocarpine 1.25% ophthalmic solution. For brand Vuity (pilocarpine hydrochloride), reauthorization updated to include therapeutic failure or intolerance to generic pilocarpine 1.25% ophthalmic solution.
Pretomanid – Commercial and Healthcare Reform	04/28/2026	Policy updated to require a member to have a diagnosis of pulmonary tuberculosis (TB) classified as rifampin-resistant or multidrug resistant (MDR) TB and for the member to experience therapeutic failure, contraindication, or intolerance to a rifamycin antibiotic. If the member has MDR TB, the member has experienced therapeutic failure, contraindication, or intolerance to isoniazid.
Proteasome Inhibitors – Commercial and Healthcare Reform	04/28/2026	Policy revised for Ninlaro (ixazomib) to require the member is 18 years of age or older.
Repository Corticotropin Injection – Commercial and Healthcare Reform – Delaware Fully Insured Commercial and Healthcare Reform	Effective upon completion of internal review and implementation	New policy for Delaware fully insured commercial and Delaware healthcare reform members for Acthar Gel Vial (repository corticotropin), Acthar Gel SelfJect (repository corticotropin), Purified Cortrophin Gel (repository corticotropin), and Purified Cortrophin Gel Prefilled Syringe (repository corticotropin). Criteria for Acthar Gel Vial (repository corticotropin) requires age of less than 2 years and diagnosis of infantile spasms.
Step Therapy Exceptions –Commercial and Healthcare Reform	03/30/2026	Policy revised to add sub-bullet under prescribing considerations. Pursuant to the Domestic & Foreign Insurers Bulletin #165 (with hyperlink), members with plans issued in Delaware may be eligible for an exception for biologics/biosimilars.
Strensiq (asfotase alfa) – Commercial and Healthcare Reform	04/28/2026	Policy revised to require, if member is ≥ 18 years of age at the time of the request, need documentation of history of onset of symptoms prior to 18 years of age.

Policy Name*	Policy Effective Date**	Updates and/or Approval Criteria
		Clarified serum alkaline phosphatase (ALP) is below the gender and age-adjusted normal range per laboratory reference range. Quantity limits section added to policy. Prescriber must document patient's weight and specify the dosing schedule. A dosing schedule of 3 mg/kg 3 times a week will only be approved for a documented diagnosis of perinatal/infantile onset hypophosphatasia (HPP). Charts have been included to determine the PLA for the vials that should be entered at the time of approval. Added to Limitations of Coverage: Patients weighing less than 40 kg cannot obtain the 80 mg/0.8 mL vial. The systemic asfotase alfa exposure achieved with the 80 mg/0.8 mL vial (higher concentration) is lower than that achieved with the other strength vials (lower concentration). A lower exposure may not be adequate for this subgroup of patients.
Tarpeyo (budesonide) – Commercial and Healthcare Reform	Termed 4/17/2026	Policy terminated and merged with J-1278.
Tarpeyo (budesonide) – Commercial and Healthcare Reform (ad-hoc)	02/19/2026	Policy revised to remove step through Filspari (sparsentan) and SGLT2i's in initial and reauthorization criteria.
Wakix (pitolisant) – Commercial and Healthcare Reform	04/28/2026	Policy revised for Wakix (pitolisant) for diagnosis of narcolepsy without cataplexy to require trial of generic modafinil and generic stimulant for patients 18 years or older or to require generic modafinil or a stimulant for patients less than 18 years of age.
Zycubo (copper histidinate) – Commercial and Healthcare Reform	04/28/2026	Policy created for Zycubo (copper histidinate) to require member is ≤ 17 years of age, the member has a documented presence of Menke disease confirmed by biochemical confirmation (elevated DOPAC:DHPG and dopamine: norepinephrine ratios or genetic confirmation of ATP7A mutation consistent with a severe Menke disease phenotype OR biochemical or genetic

Policy Name*	Policy Effective Date**	Updates and/or Approval Criteria
		results are pending and a delay in therapy would result in harm. However, documentation of clinical signs and symptoms that are strongly consistent with Menke disease are submitted for review. Also, the Zycubo is prescribed by either a pediatric neurologist, a specialist in medical genetics or another specialty with experience in treating Menke disease. For reauthorization, the presence of Menke disease must be confirmed by either biochemical or genetic confirmation and the prescriber attests that the member has experienced positive clinical response to therapy.

*For Commercial and Healthcare Reform policies, an exception to some or all the criteria above may be granted for select members and/or circumstances based on state and/or federal regulations.
**All effective dates are tentative and subject to delay pending internal review or approval.

2. Managed Prescription Drug Coverage (MRxC) Program

Policy Name*	Policy Effective Date**	Updates and Automatic Approval Criteria
Acute Migraine Therapies – Commercial and Healthcare Reform	04/28/2026	Policy revised to remove brand Imitrex (sumatriptan) nasal spray as it is off market.
Atypical Antipsychotics – Commercial	Effective upon completion of internal review and implementation	Policy revised to include Bysanti (milsaperidone) as a target medication. For the treatment of schizophrenia, the member must be 18 years of age or older, the member must have a diagnosis of schizophrenia, and the member must have experienced therapeutic failure, contraindication, or intolerance to one (1) of the following plan-preferred generic products: quetiapine product or aripiprazole tablets. For bipolar disorder, the member must be 18 years of age or older, the member must have a diagnosis of acute manic or mixed episodes associated with bipolar I disorder or bipolar II disorder, and the member must have experienced therapeutic failure, contraindication, or intolerance to one (1) of the following plan-preferred generic products: quetiapine product or aripiprazole tablets. For reauthorization the prescriber attests that

Policy Name*	Policy Effective Date**	Updates and Automatic Approval Criteria
		the member has experienced positive clinical response to therapy.
Atypical Antipsychotics – Healthcare Reform	Effective upon completion of internal review and implementation	Policy revised to include Bysanti (milsaperidone) as a target medication. For the treatment of schizophrenia, the member must be 18 years of age or older, the member must have a diagnosis of schizophrenia, and the member must have experienced therapeutic failure, contraindication, or intolerance to one (1) of the following plan-preferred generic products: quetiapine product or aripiprazole tablets. For bipolar disorder, the member must be 18 years of age or older, the member must have a diagnosis of acute manic or mixed episodes associated with bipolar I disorder of bipolar II disorder, and the member must have experienced therapeutic failure, contraindication, or intolerance to one (1) of the following plan-preferred generic products: quetiapine product or aripiprazole tablets. For reauthorization the prescriber attests that the member has experienced positive clinical response to therapy.
Beta Blocker Management – Commercial and Healthcare Reform	04/28/2026	Policy revised to add single source brand metoprolol tartrate 12.5 mg as a target, requiring diagnosis of FDA-approved indication, and therapeutic failure or intolerance to two (2) additional chemically-unique, plan-preferred, generic beta-blockers or beta-blocker combinations. Reauthorization to require positive clinical response to therapy.
Colony-Stimulating Factors – Commercial and Healthcare Reform	Effective upon completion of internal review and implementation	Policy revised for Filkri (filgrastim-laha) to require diagnosis based on FDA-approved indication, and trial/failure to Nivestym (filgrastim-aafi) and Zarxio (filgrastim-sndz) except when Filkri is being used for Hematopoietic Acute Radiation Syndrome. Filkri is to not be used in combination with other colony-stimulating factor agents. Reauthorization criteria for Filkri requires diagnosis based on FDA-approved indication and colony-stimulating factor

Policy Name*	Policy Effective Date**	Updates and Automatic Approval Criteria
		agents to not be used in combination with each other.
Concomitant Dipeptidyl Peptidase-4 (DPP-4) Inhibitors and Glucagon-Like Peptide-1 Receptor Agonist (GLP-1 RAs) Therapy - Commercial and Healthcare Reform	Effective upon completion of internal review and implementation	Policy revised to add Brynovin (sitagliptin) as a target.
Continuous Glucose Monitoring (CGM) Systems – Commercial and Healthcare Reform	04/28/2026	Policy updated to add MiniMed Instinct Sensor and Simplera as targets and policy criteria mirrors Eversense.
Cyclobenzaprine, Metaxalone, Methocarbamol, and Tizanidine Products – Commercial and Healthcare Reform	04/28/2026	Policy updated to include Tizanidine 8 mg capsules (Zanaflex authorized generic [AG]). For coverage of Tizanidine 8 mg capsules, the member must have an FDA-approved diagnosis and have experienced therapeutic failure or intolerance to either tizanidine 2 mg or 4 mg tablets and therapeutic failure or intolerance to either tizanidine 2 mg, 4 mg, or 6 mg capsules. For reauthorization, the member is experiencing a new episode of spasticity.
Doxycycline Products – Commercial and Healthcare Reform	04/28/2026	Policy revised to remove Monodox (doxycycline monohydrate) as a target as it is no longer on the market and clarified all strengths of doxycycline hyclate delayed release tablets are targets.
Extended Release Opioid Management – Commercial and Healthcare Reform	04/28/2026	Policy revised to include new product Xyvona (levorphanol) under Extended-Release Drug Products
Glucagon-Like Peptide-1 Receptor Agonists (GLP-1 RAs) for Diabetes – Commercial and Healthcare Reform	04/28/2024	Policy revised to add Ozempic (semaglutide) tablet as a target.
Herpetic Keratitis – Commercial and Healthcare Reform	04/28/2026	Policy revised for Zirgan (ganciclovir) to require age of 2 years or older and revise step through trifluridine 1% eye drops to only apply to members 6 years of age or older.
5-ASA Ulcerative Colitis Products – Commercial and Healthcare Reform	Effective upon completion of	Policy revised to add Azulfidine (sulfasalazine) and Colazal (balsalazide) as targets requiring age (Colazal), FDA-

Policy Name*	Policy Effective Date**	Updates and Automatic Approval Criteria
	internal review and implementation	approved diagnosis, and trial/failure to AB-rated generic product. Azulfidine auth duration 6 months. Colazal initial auth duration 12 weeks and reauth duration 6 months. Criteria revised for mesalamine 800 mg DR and Pentasa (mesalamine) to allow mesalamine 1,000 mg suppository as a step therapy option.
5-ASA Ulcerative Colitis Products – Commercial and Healthcare Reform	Effective upon completion of internal review and implementation	Policy revised to add Azulfidine En-tab (sulfasalazine) brand as a target requiring age, FDA-approved diagnosis, and trial/failure to AB-rated generic. Added Dipentum (olsalazine) as a target requiring, age, FDA-approved diagnosis, and trial/failure to generic sulfasalazine. Authorization duration of 6 months.
Naprosyn (naproxen) Oral Suspension, meloxicam suspension, and ketoprofen 25 mg – Commercial National Select Formulary	04/28/2026	Policy revised to add Zybic (meloxicam) suspension to require an FDA-approved diagnosis and the member has an inability to swallow meloxicam tablets and the member has experienced therapeutic failure or intolerance to generic ibuprofen suspension. Also added Orudis (ketoprofen); requires an FDA-approved diagnosis and the member must have experienced therapeutic failure or intolerance to three (3) of the following generic products, or contraindication to all of the following: diclofenac sodium/potassium tablets, ibuprofen tablets, indomethacin capsules or indomethacin extended-release capsules, meloxicam tablets, nabumetone tablets, naproxen tablets or naproxen sodium tablets. For Orudis and Zybec reauthorization, the prescriber attests that the member has experienced a positive clinical response to therapy, In addition, for Zybec, the member still has an inability to swallow meloxicam tablets. Orudis is also added to the automatic approval criteria along with Ketoprofen and Kiprofen.
Non-Preferred Atypical Antipsychotic Medications – Healthcare Reform Essential Formulary	Effective upon completion of internal review and implementation	Policy revised to include Bysanti (milsaperidone) as a target medication. For the treatment of schizophrenia, the member must be 18 years of age or older, have a diagnosis of schizophrenia, and the member

Policy Name*	Policy Effective Date**	Updates and Automatic Approval Criteria
		must have experienced therapeutic failure, contraindication, or intolerance to two (2) of the following plan-preferred generic products or all are contraindicated: risperidone, olanzapine, or quetiapine. For bipolar disorder, the member must be 18 years of age or older, must have a diagnosis of acute manic or mixed episodes associated with bipolar I disorder or bipolar II disorder, and the member must have experienced therapeutic failure, contraindication, or intolerance to two (2) of the following plan-preferred generic products, or all are contraindicated: olanzapine, quetiapine or risperidone. Automatic Approval Criteria for Bysanti occurs when there is at least one (1) paid claim for two (2) of the following antipsychotic products in the patient's drug claims history within the past 720 days: risperidone, olanzapine, or quetiapine OR there is a claim for Bysanti, Fanapt, Latuda, or Saphris in the member's prescription drug claims history within the past 180 days.
Non-Preferred Liquid Dosage Form Drugs – Commercial and Healthcare Reform	Effective upon completion of internal review and implementation	Policy revised to include Ontralfy (tizanidine), Spritam (levetiracetam), and Levetiracetam tablets for oral suspension.
Non-Preferred NSAIDs – Commercial and Healthcare Reform	04/28/2026	Policy revised to include Orudis (ketoprofen) and Zybic (meloxicam suspension) as targets. Both require diagnosis of FDA-approved indication. Orudis requires therapeutic failure, or intolerance to three (3) of the following generic products, or contraindication to all of the following: diclofenac potassium/sodium tablets, ibuprofen tablets, indomethacin capsules/extended-release capsules, meloxicam tablets, nabumetone tablets, naproxen/sodium tablets. Zybic suspension requires a diagnosis of FDA-approved indication and the member has an inability to swallow tablets or capsules and the member has experienced therapeutic failure or intolerance to plan-preferred generic ibuprofen suspension. Orudis is also added

Policy Name*	Policy Effective Date**	Updates and Automatic Approval Criteria
		to automatic approval criteria for NSAID tablets, capsules or suppositories.
Non-Preferred Selective Serotonin Reuptake Inhibitors (SSRIs) – Commercial and Healthcare Reform	04/28/2026	Policy revised to reflect the previously single-source brand Citalopram capsules is now flipped to generic designation. No policy changes.
Opioid Management – Commercial – Pennsylvania, Delaware, West Virginia	04/28/2026	Policy revised to include new product Xyvona (levorphanol) under Extended-Release opioid examples
Opioid Management – Healthcare Reform – Pennsylvania, Delaware, West Virginia	04/28/2026	Policy revised to include new product Xyvona (levorphanol) under Extended-Release opioid examples
Topical Corticosteroids – Commercial and Healthcare Reform	04/28/2026	Policy revised to add halobetasol propionate 0.05% lotion to high potency topical corticosteroids requiring therapeutic failure or intolerance to at least two plan preferred topical corticosteroids.
Trulance (plecanatide) – Commercial and Healthcare Reform	Termed 1/1/2026	Terminated 01.01.2026

*For Commercial and Healthcare Reform policies, an exception to some or all the criteria above may be granted for select members and/or circumstances based on state and/or federal regulations.
**All effective dates are tentative and subject to delay pending internal review or approval.
Standard prior authorization criteria will apply for members who do not meet the automatic approval criteria.

3. Formulary Program

Policy Name*	Policy Effective Date**	Updates and/or Approval Criteria
Clinical Exceptions for Therapeutically Equivalent Products – Commercial and Healthcare Reform	04/08/2026	New policy created for non-preferred products with therapeutic equivalent preferred products requiring applicable prior authorization policy are met, and the member has documented experiencing therapeutic failure, contraindication, or intolerance to all therapeutically equivalent preferred products documented by: 1) an adverse drug reaction (ADR) that is clinically significant and prevents continued use, supported by medical records detailing the reaction, evidence of causality (e.g., Naranjo score ≥9 or WHO-UMC Certain), and documentation of

Policy Name*	Policy Effective Date**	Updates and/or Approval Criteria
		reporting to FDA MedWatch or FAERS. 2) Documented contraindication or intolerance to an inactive ingredient (excipient) in the preferred product(s) that would result in a significant allergy or serious adverse reaction. 3) Documented evidence of treatment failure (loss of effectiveness or worsening disease) with the preferred product(s) after an adequate trial period, where medical records show objective measures of disease worsening or lack of response, and the prescriber has assessed that this failure was attributable to the ineffectiveness of the therapeutically equivalent preferred product(s) and not primarily due to factors such as the standard course of the disease, known disease triggers, non-adherence, co-morbid conditions, or the patient being a non-responder or developing resistance to the active ingredient.
Experimental and Investigational Products – Commercial and Healthcare Reform	Effective upon completion of internal review and implementation	Policy revised to add Acthar Gel Vial (repository corticotropin) for members 2 years of age or older, Acthar Gel SelfJect (repository corticotropin), and Purified Cortrophin Gel (repository corticotropin). No exceptions will be made for experimental and investigational products.
Market Watch Programs – Delaware	Effective upon completion of internal review and implementation	Policy revised for high-cost low value to add: 1) Pokonza (potassium chloride) oral solution to step through potassium chloride capsule, potassium chloride tablet, and potassium chloride 20 mEq packet; 2) Sdamlo (amlodipine) solution to step through amlodipine tablet; 3) Lopressor (metoprolol tartrate) 12.5 mg and metoprolol tartrate 12.5 mg to step through metoprolol tartrate 25 mg tablet [can be split]; 4) Nereus (tradipitant) to step through meclizine and scopolamine patch. 5) Zybic (meloxicam) oral suspension to step through ibuprofen suspension and celecoxib capsules. 6) Orudis (ketoprofen) to step through naproxen tablet, indomethacin ER capsule, diclofenac sodium tablet, and

Policy Name*	Policy Effective Date**	Updates and/or Approval Criteria
		meloxicam tab; 7) Tizanidine 8 mg capsule to step through tizanidine 2 mg, 4 mg tablets and tizanidine 2 mg, 4 mg, 6 mg capsules; 8) Xyvona (levorphanol) to step through Morphine Sulfate ER tablet; 9) Spritam (levetiracetam) and levetiracetam tablet for oral suspension to step through levetiracetam tablet and oral solution.
Market Watch Programs – New York, Pennsylvania and West Virginia	Effective upon completion of internal review and implementation	Policy revised for high-cost low value to add: 1) Pokonza (potassium chloride) oral solution to step through potassium chloride capsule, potassium chloride tablet, and potassium chloride 20 mEq packet; 2) Sdamlo (amlodipine) solution to step through amlodipine tablet; 3) Lopressor (metoprolol tartrate) 12.5 mg and metoprolol tartrate 12.5 mg to step through metoprolol tartrate 25 mg tablet [can be split]; 4) Nereus (tradipitant) to step through meclizine and scopolamine patch. 5) Zybic (meloxicam) oral suspension to step through ibuprofen suspension and celecoxib capsules. 6) Orudis (ketoprofen) to step through naproxen tablet, indomethacin ER capsule, diclofenac sodium tablet, and meloxicam tab; 7) Tizanidine 8 mg capsule to step through tizanidine 2 mg, 4 mg tablets and tizanidine 2 mg, 4 mg, 6 mg capsules; 8) Xyvona (levorphanol) to step through Morphine Sulfate ER tablet; 9) Spritam (levetiracetam) and levetiracetam tablet for oral suspension to step through levetiracetam tablet and oral solution.

*For Commercial and Healthcare Reform policies, an exception to some or all the criteria above may be granted for select members and/or circumstances based on state and/or federal regulations.
**All effective dates are tentative and subject to delay pending internal review or approval.

4. Quantity Level Limit (QLL) Programs*

Table 1. Quantity Level Limits – Quantity per Duration for Commercial and Healthcare Reform Plans

Drug Name	Retail Edit Limit	Mail Edit Limit
Bysanti (milsaperidone) Titration Packs	1 titration pack per 365 days	1 titration pack per 365 days

Drug Name	Retail Edit Limit	Mail Edit Limit
Desmoda (desmopressin acetate)	1 bottle (145 mL) per 30 days	3 bottles (435 mL) per 90 days
Nereus (tradipitant)	4 tablets per 365 days	4 tablets per 365 days
Ontralfy (tizanidine oral solution)	2,790 mL/30 days	8,370 mL/90 days
Strensiq (asfotase alfa) injection 18 mg/0.45 mL, 28 mg/0.7 mL, 40 mg/mL	1 vial/28 days	4 vials/28 days
Strensiq (asfotase alfa) injection 80 mg/0.8 mL	2 vials/28 days	8 vials/28 days
Yuviwel (navepegritide) 1.3 mg & 2.8 mg vials	4 kits (2.4 mL) per 28 days	12 kits (7.2 mL) per 84 days
Yuviwel (navepegritide) 5.5 mg vial	8 kits (8 mL) per 28 days	24 kits (24 mL) per 84 days
Zepbound (tirzepatide) Kwikpen	1 pen per 21 days	3 pens per 63 days

Table 2. Quantity Level Limits – Quantity per Dispensing Event – Commercial and Healthcare Reform Plans

Drug Name	Retail Edit Limit	Mail Edit Limit
Adquey (difamilast)	1 tube (27 or 85 grams) per dispensing event	3 tubes (81 or 255 grams) per dispensing event
Bimatoprost PF (brand Single Source)	1 bottle (5 mL) per dispensing event	3 bottles (15 mL) per dispensing event
Pivya (pivmecillinam)	21 tablets per dispensing event	21 tablets per dispensing event

Quantity per dispensing event limits the quantity of medication that can be dispensed per each fill. If the submitted day supply on a claim is 34 days or less, the retail limit will apply. If the submitted day supply on a claim is greater than 34 days, the mail limit will apply.

Table 3. Maximum Daily Quantity Limits – Commercial and Healthcare Reform Plans

Drug Name	Daily Limit
Bysanti (milsaperidone)	2 tablets per day
Juxtapid (lomitapide) 2 mg capsule	1 capsule per day
Prezcobix PED (darunavir and cobicistat)	1 tablet per day
Seysara (sarecycline)	1 tablet per day
Yuvezzi (carbachol and brimonidine tartrate ophthalmic solution)	1 single dose vial per day
Zycubo (copper histidinate)	one 2.9 mg vial/day

*Quantity per Duration (QD) rule also applies to this medication (refer to Table 1).
Members can receive up to the maximum day supply according to their benefits, but the daily limit must not be exceeded for each individual day.

Requests for coverage of select medications exceeding the defined quantity level limits may be submitted for clinical review. Maximum-day supply on certain medications may vary depending on member's benefit design.**SECTION II. Highmark Medicare Part D Formularies**

A. Changes to the Highmark Medicare Part D 5-Tier Open Formularies

The Highmark Pharmacy and Therapeutics Committee has reviewed the medications listed in the tables below. For your convenience, you can search the Highmark Medicare Part D Formularies online at:

- **Incentive Formulary**
- **Compass Formulary**

Table 1. Preferred Products

Effective immediately pending Centers for Medicare and Medicaid Services (CMS) approval and upon completion of internal review and implementation.

No changes at this time.

Table 2. Non-Preferred Products

Effective immediately pending CMS approval and upon completion of internal review and implementation.

Brand Name	Generic Name	Preferred Alternatives
Adquey	difamilast	tacrolimus ointment, triamcinolone acetonide cream
Bimatoprost PF	Bimatoprost PF	Prescriber discretion
Bysanti Titration Pack	milsaperidone	olanzapine, quetiapine, risperidone
Nereus	tradipitant	meclizine, scopolamine base
Ontralfy	tizanidine oral solution	tizanidine tablets
Yuvezzi	carbachol and brimonidine tartrate ophthalmic solution	Prescriber discretion

B. Changes to the Highmark Medicare Part D 5-Tier Closed Formulary

The Highmark Pharmacy and Therapeutics Committee has reviewed the medications listed in the tables below. For your convenience, you can search the Highmark Medicare Part D Formularies online at:

- **Performance Formulary**
- **Venture Formulary**
- **Fundamental Formulary**

Table 1. Preferred Products

Effective immediately pending CMS approval and upon completion of internal review and implementation.

No changes at this time.

Table 2. Non-Preferred Products
Effective immediately pending CMS approval and upon completion of internal review and implementation.

Brand Name	Generic Name	Preferred Alternatives
Bysanti Titration Pack	milsaperidone	olanzapine, quetiapine, risperidone

Table 3. Products Not Added*
Effective immediately pending CMS approval and upon completion of internal review and implementation.

Brand Name	Generic Name	Preferred Alternatives
Adquey	difamilast	tacrolimus ointment, triamcinolone acetonide cream
Avopef	etoposide	Prescriber discretion
Bimatoprost PF	Bimatoprost PF	Prescriber discretion
Desmoda	desmopressin acetate	desmopressin tablets
Filkri	filgrastim-laha	Nivestym, Zarxio
Nereus	tradipitant	meclizine, scopolamine base
Ontralfy	tizanidine oral solution	tizanidine tablets
Yartemlea	narsoplimab-wuug	Prescriber discretion
Yuvezzi	carbachol and brimonidine tartrate ophthalmic solution	Prescriber discretion
Yuviwel	navepegritide	Prescriber discretion

*Physicians may request coverage of these products using the Prescription Drug Medication Request Form.

C. Additions to the Specialty Tier

Effective immediately pending CMS approval and upon completion of internal review and implementation.

Brand Name	Generic Name
Avopef*	etoposide
Bysanti	milsaperidone
Desmoda*	desmopressin acetate
Filkri*	filgrastim-laha
Loargys	pegzilarginase-nbln
Pokonza oral solution	potassium chloride
Prezcobix PED	darunavir and cobicistat
Yartemlea*	narsoplimab-wuug
Yuviwel*	navepegritide
Zycubo	copper histidinate

*Only pertains to Incentive and Compass formularies

D. Updates to the Pharmacy Utilization Management Programs

1. Prior Authorization Program

Policy Name	Policy Effective Date*	Updates and/or Approval Criteria
Achondroplasia – Medicare	04/28/2026	Policy revised to add Yuviwel (navepegritide) as a target requiring pediatric age, genetic confirmation of achondroplasia, baseline annualized growth velocity (AGV), and open epiphyses. Reauthorization requiring an increase in AGV compared to baseline and open epiphysis.
Administrative Prior Authorizations for Medicare Part D Plans – Medicare	04/28/2026	Policy updated to add products: Bosaya (denosumab-kyqq), Enoby (denosumab-qbde), and Xtrenbo (denosumab-qbde) to II. BvD section H Injectable Osteoporosis Agents, Gammagard ERC and Qivigy to II. BvD section B. Intravenous Immune Globulins (IVIG), and baclofen, Gablofen (baclofen), Lioresal (baclofen) to II. BvD section L. Drugs Administered via Infusion Pump
Adquey (difamilast) Vtama (tapinarof) and Zoryve (roflumilast) – Medicare	Effective upon completion of internal review and implementation	Policy revised to add Adquey (difamilast) for atopic dermatitis requiring FDA-approved diagnosis, failure/intolerance/contraindication to one generic formulary topical corticosteroid or facial or anogenital involvement, and failure/intolerance/contraindication to generic topical tacrolimus or pimecrolimus.
ALK-Targeting Kinase Inhibitors – Medicare	04/28/2026	Policy revised for Ensacove (ensartinib) to require that disease is ALK-positive, as detected by an FDA-approved test.
Amyloid beta-Directed Antibodies for Alzheimer's Disease – Medicare	03/27/2026	Policy revised to remove confirmation of Positron Emission Tomography (PET) imaging for amyloid beta pathology on initial and reauthorization.
Amyloid beta-Directed Antibodies for Alzheimer's Disease – Medicare	03/03/2026	Policy revised for Leqembi Iqlik (lecanemab-irmb) for to require that the member has completed or is receiving or undergoing 18 months of intravenous (IV) infusions of Leqembi. For reauthorization, the member has experienced reduction from baseline in amyloid beta plaque levels via PET imaging or has demonstrated positive clinical response

Policy Name	Policy Effective Date*	Updates and/or Approval Criteria
		as evidenced by slowed decline in cognition.
Anti-EGFR and HER2 Kinase Inhibitors – Medicare	04/28/2026	Policy revised for Hernexeos (zongertinib) to remove requirement of prior systemic therapy based on updated FDA-approved indication for non-small cell lung cancer.
Antifibrotic Pulmonary Medications – Medicare	02/03/2026	Policy revised for Jascayd (nerandomilast) in the treatment of idiopathic pulmonary fibrosis (IPF) to change the criterion requiring a baseline forced vital capacity (FVC) from at least 50% to at least 45%.
Antifibrotic Pulmonary Medications – Medicare	03/27/2026	Policy revised to address UMGD concern for Jascayd used for diagnosis of progressive pulmonary fibrosis (PFF). Two (2) clinical signs of progression are required. The definition of PPF and the criteria required are taken from the ATS/ERS/JRS/ALAT Clinical Practice Guideline. For coverage of Jascayd in the treatment of PPF the following criteria must be met: 1) The member has a diagnosis of PFF/ILD with Progressive Phenotype. 2) There is documentation of a chest computed tomography (CT) scan demonstrating ≥ 10% pulmonary fibrosis, 3) The member has a baseline forced vital capacity (FVC) of at least 45%. 4) The member has a percent predicted diffusing capacity of the lungs of carbon monoxide (DLCO) of at least 25%. 5) The member presents with clinical signs of progression defined as two (2) of the following three (3) criteria (a., b., c.): a) Worsening respiratory symptoms b) One of the following physiological signs of disease progression (i. or ii.): i) Absolute decline in FVC ≥ 5% predicted within 1 year of follow-up.ii) Absolute decline in DLCO (corrected for hemoglobin) ≥ 10% predicted within 1 year of follow-up. c) Presence of one (1) or more of the following radiological evidence of disease progression (i. through vi.): i. Increased extent or severity of traction bronchiectasis and bronchiolectasis ii.

Policy Name	Policy Effective Date*	Updates and/or Approval Criteria
		New ground-glass opacity with traction bronchiectasis iii. New fine reticulation iv. Increased extent or increased coarseness of reticular abnormality v. New or increased honeycombing vi. Increased lobar volume loss 6. The member meets one (1) of the following criteria (a., b., or c.): a) The member has been on stable therapy with Ofev and will continue Ofev concomitantly with Jayscayd. b)The member is treatment naïve to antifibrotic treatment. c) The member previously discontinued Ofev and does not plan to start or re-start background PPF treatment. In addition, to address an UMGD concern, the Jascayd criteria for idiopathic pulmonary fibrosis (IPF) was revised to change the criterion requiring a baseline forced vital capacity (FVC) from at least 50% to at least 45%.
Atypical Antipsychotics – Medicare	Effective upon completion of internal review and implementation	Policy revised to include Bysanti (milsaperidone) as a target medication. For the treatment of schizophrenia the member must have a diagnosis of schizophrenia, and the member must have experienced therapeutic failure, contraindication, or intolerance to one (1) other generic atypical antipsychotic, (for example: quetiapine). For bipolar disorder, the member must have a diagnosis of acute manic or mixed episodes associated with bipolar I disorder or bipolar II disorder, and the member must have experienced therapeutic failure, contraindication, or intolerance to one (1) of the following generic products: olanzapine, quetiapine or risperidone.
Bispecific Monoclonal Antibodies for Multiple Myeloma – Medicare	04/28/2026	Policy revised for Tecvayli (teclistamab-cgyv) to require diagnosis based on updated indication for use as monotherapy in multiple myeloma and new indication for use in combination with daratumumab hyaluronidase-fihj in multiple myeloma.

Policy Name	Policy Effective Date*	Updates and/or Approval Criteria
BRAF Mutation-Targeting & MEK Kinase Inhibitors – Medicare	04/28/2026	Policy revised for Braftovi (encorafenib) to require diagnosis based on updated indication for metastatic colorectal cancer used in combination with cetuximab and fluorouracil-based chemotherapy.
Chronic Inflammatory Diseases – Medicare	02/27/2026	Policy revised for Rinvoq (upadacitinib) and tocilizumab products to remove systemic corticosteroid step from giant cell arteritis criteria.
Chronic Inflammatory Diseases – Medicare	Effective upon completion of internal review and implementation	Policy revised for Sotyktu (deucravacitinib) to add new indication of psoriatic arthritis (PsA) requiring the member is 18 years or older and step through 2 preferred agents for PsA. Policy revised to add Avtozma SC (tocilizumab-anoh) to have criteria matching existing tocilizumab SC products and for Cosentyx (secukinumab) to update hidradenitis suppurativa age restriction to 12 years of age and older.
Chronic Inflammatory Diseases – Medicare	02/09/2026	Policy revised for Xeljanz (tofacitinib) to broaden age restriction to include adults. Policy revised for Rinvoq (upadacitinib) in giant cell arteritis to include that Rinvoq will be used in combination with or without a systemic corticosteroid.
Clemastine Fumarate Oral Syrup and Corphena (dexchlorpheniramine maleate) – Medicare	04/28/2026	Policy updated to include Corphena (dexchlorpheniramine maleate) to require that the member has an FDA-approved diagnosis; has experienced therapeutic failure, contraindication, or intolerance to two (2) generic oral antihistamine products
Darzalex (daratumumab) and Darzalex Faspro (daratumumab and hyaluronidase-fihj) – Medicare	04/28/2026	Policy revised for Darzalex Faspro (daratumumab and hyaluronidase) to require criteria based on expanded FDA-approved indication for newly diagnosed patients with multiple myeloma ineligible for autologous stem cell transplant.
Denosumab Products for Bone Disease and Evenity (romosozumab-aqqg) – Medicare	04/17/2026	Policy revised to add Evenity (romosozumab) as a target requiring criteria previously approved in J-0922. Definitions of high and very high fracture risk to match American Association of Clinical Endocrinologists/American College of Endocrinology. For

Policy Name	Policy Effective Date*	Updates and/or Approval Criteria
		denosumab products, patients at very high fracture risk do not have to try/fail bisphosphonates.
Desmoda (desmopressin acetate) – Medicare	04/28/2026	New policy created for Desmoda (desmopressin acetate), initial authorization criteria requires diagnosis, the member has an intolerance to generic desmopressin acetate tablets, and has an inability to swallow tablets. For reauthorization, the prescriber attests that the member has experienced positive clinical response to therapy, the member has experienced an intolerance to generic desmopressin acetate tablets, and has an inability to swallow tablets. Limitations of coverage added requiring the member meets all of the following creatine clearance is ≥ 50 mL/min and serum sodium concentration is ≥ 135 mEq/L. Initial authorization duration of 6 months, and reauthorization duration of 12 months.
Dichlorphenamide Products – Medicare	04/28/2026	Policy revised to require an intolerance to generic dichlorphenamide (generic Keveyis) prior to coverage of brand Keveyis or Ormalvi. Removed trial and failure from the criterion.
Dichlorphenamide Products – Medicare	02/26/2026	Policy revised for Keveyis (dichlorphenamide) and Ormalvi (dichlorphenamide) to remove "weekly" and "from baseline" requirements from number of muscle weakness attacks in reauthorization criteria.
Dupixent (dupilumab) – Medicare	Effective upon completion of internal review and implementation	Policy revised for Dupixent (dupilumab) for new indication of allergic fungal rhinosinusitis (AFRS) to require FDA-approved diagnosis, and diagnosis confirmed by two of the following: Type 1 hypersensitivity to fungus confirmed by history, in vitro IgE immunoassays, or skin testing; positive fungal stain of sinus contents removed during surgery; presence of eosinophilic mucin without invasion into the sinus tissue; presence of chronic rhinosinusitis with nasal polyposis (NP), or characteristic CT signs.

Policy Name	Policy Effective Date*	Updates and/or Approval Criteria
		Reauthorization requires decrease in nasal polyp score or nasal congestion/obstruction severity score.
Dupixent (dupilumab) – Medicare	02/09/2026	Criteria revised for Dupixent for atopic dermatitis to include that Dupixent will be used combination with or without a generic topical corticosteroid.
Enjaymo (sutimlimab-jome) – Medicare	04/28/2026	Policy revised for Enjaymo (sutimlimab-jome) to include quantity chart for induction and maintenance dosing.
Enzyme Replacement Therapy for Gaucher Disease – Medicare	04/28/2026	Policy revised for Cerezyme (imiglucerase) to add expanded indication of Type 3 Gaucher disease.
Evenity (romosozumab-aqqg) – Medicare	Termed 04/17/2026	Policy terminated (combined with J-0134).
Glucagon-Like Peptide-1 Receptor Agonists (GLP-1 RAs) – Medicare	Effective upon completion of internal review and implementation	Policy revised to add Ozempic (semaglutide) tablet as a target mirroring criteria for Rybelsus (semaglutide).
Homozygous Familial Hypercholesterolemia – Medicare	04/28/2026	Initial authorization criteria for Evkeeza (evinacumab-dgnb) and Juxtapid (lomitapide) was revised to the member meets one of the following LDL-C > 560 mg/dL or LDL-C > 400 mg/dL and meets one of the following: aortic valve disease or xanthomata at < 20 years of age, one or both parents having clinically diagnosed HoFH, positive genetic testing for a known LDL-C–raising (LDLR, Apo[b], PCSK9) gene defect, or autosomal-recessive familial hypercholesterolemia. Criteria also now requires if the member is 10 years of age or older, the member has experienced therapeutic failure, contraindication, or intolerance to Repatha (evolocumab). Additionally, for Juxtapid initial authorization criteria was revised to account for new age expansion. Criteria now requires an LDL > 115 mg/dL for members 17 years of age or younger, or LDL > 70 mg/dL if 18 years of age or older, or LDL > 55 mg/dL if 18 years of age and older and atherosclerotic cardiovascular disease (ASCVD) or major ASCVD risk factors.

Policy Name	Policy Effective Date*	Updates and/or Approval Criteria
Human Growth Hormone – Medicare	02/26/2026	Policy revised for treatment of HIV wasting to allow approval for 5% weight loss from baseline in the past 6 months.
Human Growth Hormone – Medicare	Effective upon completion of internal review and implementation	Policy revised for Sogroya (somapacitan-beco) to allow approval for expanded indication for treatment of short stature and small for gestational age, idiopathic short stature, or have growth failure due to Noonan syndrome, based on previously approved criteria for these indications.
Immune Globulin - Medicare (ad-hoc)	02/09/2026	Administrative changes only to remove therapeutic failure language.
Immunoglobulin A Nephropathy (IgAN) Therapies – Medicare	02/26/2026	Policy revised for Filspari (sparsentan) to remove limitation of coverage that concurrent use with angiotensin converting enzyme inhibitors would not be approved.
Inbrija (levodopa) – Medicare	04/28/2026	Policy revised to remove age requirement.
Interleukin (IL)-5 Antagonists – Medicare (ad-hoc)	01/06/2026	Policy revised for Exdensur (depemokimab-ulaa) to require the member to experience therapeutic failure, intolerance, or contraindication to two (2) of the following products: Dupixent (dupilumab), Fasenra (benralizumab), or Nucala (mepolizumab).
Journavx (suzetrigine) – Medicare	04/28/2026	Policy revised to include revised indication: moderate-to-severe acute pain, including postoperative pain, in adults. No changes to criteria necessary.
Lumryz (sodium oxybate), Xyrem (sodium oxybate) and Xywav (calcium, magnesium, potassium, and sodium oxybates) – Medicare	Effective upon completion of internal review and implementation	Policy revised to require that the member has experienced therapeutic failure, contraindication, or intolerance to generic sodium oxybate for requests for Lumryz (sodium oxybate) or Xywav (calcium, magnesium, potassium, and sodium oxybates).
Lumryz (sodium oxybate), Xyrem (sodium oxybate) and Xywav (calcium, magnesium, potassium, and sodium oxybates) – Medicare	Effective upon completion of internal review and implementation	Policy revised to remove authorized generic sodium oxybate as it is off market. Generic of Xyrem (sodium oxybate) added to policy as target. Policy revised to change step through modafinil and stimulant to modafinil or simulant. Policy revised to add if the request is for brand Xyrem (sodium oxybate), the

Policy Name	Policy Effective Date*	Updates and/or Approval Criteria
		member has experienced intolerance to generic sodium oxybate.
Miscellaneous Immunomodulators – Medicare	Effective upon completion of internal review and implementation	Policy revised for brand Pomalyst (pomalidomide) to require intolerance to generic pomalidomide.
Mitapivat Products – Medicare	03/30/2026	Policy revised for Aqvesme (mitapivat) reauthorization criteria to require improvement in laboratory results, improvement in clinical status, or there is another explanation for response failure (e.g., bleeding, surgery, other concomitant illnesses).
Nereus (tradipitant) – Medicare	Effective upon completion of internal review and implementation	New policy for Nereus (tradipitant) requiring FDA approved diagnosis and therapeutic failure, contraindication, or intolerance to generic scopolamine patch and one generic first generation antihistamine (for example, meclizine).
Neurontin (gabapentin) – Medicare	Effective upon completion of internal review and implementation	Policy revised for brand Neurontin (gabapentin) to require intolerance to generic gabapentin immediate release tablets or capsules.
Non-preferred Oral Potassium Chloride Products – Medicare	04/28/2026	Policy revised to add Pokonza oral solution (potassium chloride) as a target mirroring criteria for Pokonza pouches for oral solution (potassium chloride).
Noxafil (posaconazole) – Medicare	Effective upon completion of internal review and implementation	Policy updated to remove age criteria for Noxafil PowderMix for delayed-release oral suspension (posaconazole) for Aspergillus or Candida Infection Prophylaxis. The weight was also updated to 10 to 40 kg. If the member has a diagnosis of invasive aspergillosis infection, the member has experienced therapeutic failure, contraindication, or intolerance to voriconazole and the member is unable to swallow tablets or capsules.
Palynziq (pegvaliase-pqpz) – Medicare	04/28/2026	Policy revised for Palynziq (pegvaliase-pqpz) to remove age limitation.
Parathyroid Hormone Analogs – Medicare	Effective upon completion of internal review and implementation	Policy revised to update definitions of high and very high fracture risk to match American Association of Clinical Endocrinologists/American College of Endocrinology.

Policy Name	Policy Effective Date*	Updates and/or Approval Criteria
PCSK9 Therapies – Medicare	04/28/2026	Policy revised for Leqvio's (inclisiran) expanded indication Heterozygous Familial Hypercholesterolemia (HeFH) and new indication Homozygous Familial Hypercholesterolemia (HoFH). Policy criteria mirrors Praluent (alirocumab) for these indications, with the exception of having an age requirement of at least 12 years old but less than 18 years old for HoFH.
PCSK9 Therapies – Medicare	02/26/2026	Policy revised for Hypercholesterolemia with ASCVD and Major Cardiovascular Risk indications to now require the member has a current LDL-C ≥ 55 mg/dL, or the member has a current non-HDL-C ≥ 85 mg/dL.
PCSK9 Therapies – Medicare	02/23/2026	Policy revised to clarify that diagnosis of homozygous familial hypercholesterolemia can be made based on genetic variation of two mutant alleles, LDL-C > 400 mg/dL, and autosomal recessive familial hypercholesterolemia.
Presbyopia Products – Medicare	Effective upon completion of internal review and implementation	Policy revised to add Yuvezzi (carbachol and brimonidine tartrate) requiring diagnosis of presbyopia and therapeutic failure, intolerance, or contraindication to generic pilocarpine hydrochloride 1.25% ophthalmic solution.
Pretomanid – Medicare	04/28/2026	Policy updated to require a member to have a diagnosis of pulmonary tuberculosis (TB) classified as rifampin-resistant or multidrug resistant (MDR) TB and for the member to experience therapeutic failure, contraindication, or intolerance to a rifamycin antibiotic. If the member has MDR TB, the member has experienced therapeutic failure, contraindication, or intolerance to isoniazid.
Programmed Death Receptor Therapies – Medicare	04/28/2026	Policy revised for Keytruda (pembrolizumab) and Keytruda Qlex (pembrolizumab and berahyaluronidase alfa-pmph) to require criteria based on expanded FDA-approved indication for

Policy Name	Policy Effective Date*	Updates and/or Approval Criteria
		epithelial ovarian, fallopian tube, or primary peritoneal carcinoma.
Pulmonary Hypertension – Medicare	03/30/2026	Policy revised to allow for one of the following for PH-ILD: right heart catheterization substantiating all of the following mean pulmonary arterial pressure > 20 mmHg at rest, pulmonary wedge pressure ≤ 15 mmHg, and pulmonary vascular resistance ≥ 3 Wood unit, or if the member is not a candidate for a right heart catheterization the member meets all of the following criteria, the risk of right heart catheterization outweighs the benefit (for example, risk of pneumothorax, fragile lung parenchyma) and results of a non-invasive test demonstrate the diagnosis of PH-ILD.
Steroidogenesis Inhibitors – Medicare	Effective upon completion of internal review and implementation	Policy revised for all targets to require FDA-approved indication for reauthorization.
Steroidogenesis Inhibitors – Medicare	02/19/2026	Policy revised for Isturisa (osilodrostat) and Recorlev (levoketoconazole) to require trial, failure, or contraindication to surgery.
Strensiq (asfotase alfa) – Medicare	Effective upon completion of internal review and implementation	Policy revised to clarify that the serum alkaline phosphatase (ALP) level must be below the gender and age-adjusted normal range per the laboratory reference range and if the request is for a member ≥ 18 years of age, the prescriber attests that the onset of signs and symptoms of hypophosphatasia (HPP) occurred prior to 18 years of age. Limitations of Coverage: Patients weighing less than 40 kg cannot obtain the 80 mg/0.8 mL vial. The systemic asfotase alfa exposure achieved with the 80 mg/0.8 mL vial (higher concentration) is lower than that achieved with the other strength vials (lower concentration). A lower exposure may not be adequate for this subgroup of patients.
Sublingual Immunotherapies – Medicare	04/28/2026	Policy revised to remove Oralair (Sweet Vernal, Orchard, Perennial Rye, Timothy, and Kentucky Blue Grass Mixed Pollens

Policy Name	Policy Effective Date*	Updates and/or Approval Criteria
		Allergen Extract) as a target due to no longer being a Medicare Part D eligible product.
Tarpeyo (budesonide) – Medicare	Termed 4/17/2026	Policy terminated and merged with J-1279.
Testosterone (Androgens) – Medicare	02/09/2026	Policy revised for all targets to remove weight loss due to HIV as an allowable diagnosis.
Testosterone (Androgens) – Medicare	Effective upon completion of internal review and implementation	Policy revised for Xyosted (testosterone enanthate) to require intolerance/contraindication to one injectable generic testosterone product.
Tizanidine – Medicare	04/28/2026	Policy updated to include Tizanidine 8 mg capsules (Zanaflex authorized generic [AG]) and Ontralfy, a tizanidine oral solution. For Tizanidine 8 mg capsules, the member must have an FDA approved diagnosis and have experienced intolerance to either tizanidine 2 mg or 4 mg tablets and intolerance to either tizanidine 2 mg, 4 mg, or 6 mg capsules. For reauthorization of Tizanidine 8 mg capsules, the member is experiencing a new episode of spasticity. For coverage of Ontralfy (tizanidine), the member must have an FDA-approved diagnosis and have an inability to swallow tizanidine tablets or capsules. For reauthorization of Ontralfy, the member is experiencing a new episode of spasticity and is still unable to swallow tizanidine tablets or capsules.
Urea Cycle Disorder Medications – Medicare	4/28/206	Policy revised to add Loargys (pegzilarginase-nbln) to require a diagnosis of Arginase 1 Deficiency confirmed by one of the following: newborn screening, blood (plasma) test for amino acids that shows elevated arginine levels, or genetic testing and the member will use Loargys in conjunction wtih dietary protein restriction.
Wegovy (semaglutide) and Zepbound (tirzepatide) – Medicare	03/30/2026	Policy revised for Wegovy (semaglutide) for treatment of metabolic dysfunction–associated steatohepatitis to remove reauthorization requirement for improvement or stabilization of fibrosis.

Policy Name	Policy Effective Date*	Updates and/or Approval Criteria
Wegovy (semaglutide) and Zepbound (tirzepatide) – Medicare	02/27/2026	Policy revised for Wegovy (semaglutide) for cardiovascular risk reduction to remove step through Ozempic (semaglutide) if the patient has type 2 diabetes.
Wegovy (semaglutide) and Zepbound (tirzepatide) – Medicare	Effective upon completion of internal review and implementation	Policy revised for Zepbound (tirzepatide) single-dose injection to require contraindication to Zepbound Kwikpen.
Wegovy (semaglutide) and Zepbound (tirzepatide) – Medicare	02/17/2026	Policy revised for Wegovy (semaglutide) for treatment of metabolic dysfunction–associated steatohepatitis to remove time requirements on diagnostic testing for initial authorization. For initial and reauthorization, attestation that the member does not have hepatic decompensation or cirrhosis.
Yartemlea (narsoplimab-wuug) – Medicare	Effective upon completion of internal review and implementation	Policy created for Yartemlea (narsoplimab-wuug) to require FDA approved diagnosis confirmed by renal or gastrointestinal biopsy or clinically using modified Jodele criteria, and member is classified as high-risk defined as elevated soluble C5b-9, urine protein-to-creatinine ratio (rUPCR) ≥ 1 mg/mg, elevated lactate dehydrogenase (LDH) ≥ 2 times ULN, grade II-IV acute GVHD, infections (viral or bacterial), or organ dysfunction. Reauthorization requires one of the following: transfusion independence, improvement in organ function, improvement in anemia, improvement in thrombocytopenia, decrease in measure of schistocytes, decrease LDH, or reduction in soluble C5b-9.
Yorvipath (palopegteriparatide) – Medicare	02/26/2026	Policy revised for Yorvipath (palopegteriparatide) to remove criteria requiring previous treatment with calcium.
Zycubo (copper histidinate) – Medicare	04/28/2026	Policy created for Zycubo (copper histidinate) to require member is ≤ 17 years of age, the member has a documented presence of Menke disease confirmed by biochemical confirmation (elevated DOPAC:DHPG and dopamine: norepinephrine ratios or genetic confirmation of ATP7A mutation

Policy Name	Policy Effective Date*	Updates and/or Approval Criteria
		consistent with a severe Menke disease phenotype OR biochemical or genetic results are pending and a delay in therapy would result in harm. However, documentation of clinical signs and symptoms that are strongly consistent with Menke disease are submitted for review. Also, the Zycubo is prescribed by either a pediatric neurologist, a specialist in medical genetics or another specialty with experience in treating Menke disease. For reauthorization, the presence of Menke disease must be confirmed by either biochemical or genetic confirmation and the prescriber attests that the member has experienced positive clinical response to therapy.

*All effective dates are tentative and subject to delay pending internal review or approval.

2. Updates to Step Therapy

Policy Name	Policy Effective Date*	Updates and/or Approval Criteria
Colony-Stimulating Factors – Medicare	Effective upon completion of internal review and implementation	Policy revised for Filkri (filgrastim-laha) to require diagnosis based on FDA-approved indication, and trial/failure to Nivestym (filgrastim-aafi) and Zarxio (filgrastim-sndz) except when Filkri is being used for Hematopoietic Acute Radiation Syndrome. Filkri is to not be used in combination with other colony-stimulating factor agents. Reauthorization criteria for Filkri requires diagnosis based on FDA-approved indication and colony-stimulating factor agents to not be used in combination with each other.
Herpetic Keratitis – Medicare	04/28/2026	Policy revised for Zirgan (ganciclovir ophthalmic gel) to only require therapeutic failure, contraindication, or intolerance to trifluridine 1% eye drops for members 6 years of age or older.
Intravitreal Injections – Medicare	Effective upon completion of internal review	Policy revised for Enzeevu (aflibercept-abzv) to add expanded indications of Macular Edema Following Retinal Vein Occlusion (MEfRVO), Diabetic Macular Edema (DME), and Diabetic

Policy Name	Policy Effective Date*	Updates and/or Approval Criteria
	and implementation	Retinopathy requiring that the member has experienced therapeutic failure after an adequate trial, contraindication, or intolerance to Avastin (bevacizumab) for a diagnosis of Neovascular (Wet) Age-Related Macular Degeneration (nAMD) and has experienced therapeutic failure or intolerance to Eylea (aflibercept).
Non-Preferred ADHD Step Therapy – Medicare	02/03/2026	Policy revised to include generic dextroamphetamine extended release.
Ophthalmic Prostaglandins and Rho Kinase Inhibitors – Medicare	02/20/2026	Policy revised to remove iDose TR (travoprost intracameral implant).
Pancreatic Enzymes – Medicare	04/02/2026	Policy revised to remove Zenpep (pancrelipase) as a target and update step therapy requirement to include Zenpep as an option.

*All effective dates are tentative and subject to delay pending internal review or approval.

3. Formulary Program

Policy Name	Policy Effective Date*	Updates and/or Approval Criteria
Blood Glucose Testing Products – Medicare	02/05/2026	Policy revised to add additional preferred products HUMANA TRUE METRIX AIR METER, TRUE METRIX GO GLUCOSE METER, HUMANA TRUE METRIX TEST STRIP, TRUETRACK GLUCOSE TEST STRIPS
Non-Formulary Requests (12 Month Authorization) – Medicare	04/28/2026	Criteria revised to clarify that trial and failure of multiple products with the same active ingredient will be accepted.
Non-Formulary Requests (plan-year authorization) – Medicare	04/28/2026	Criteria revised to clarify that trial and failure of multiple products with the same active ingredient will be accepted.

*All effective dates are tentative and subject to delay pending internal review or approval.

4. Quantity Level Limit (QLL) Program

Effective date pending CMS approval, completion of internal review and implementation, unless otherwise noted.

Drug Name	Retail Quantity Limit (31 days)
Adquey (difamilast)	85 grams per 28 days
Bimatoprost PF (Bimatoprost PF)	5 ml per 31 days
Bysanti (milsaperidone)	2 tablets per day
Bysanti (milsaperidone) Titration Pack A	Two titration packs per 365 days
Bysanti (milsaperidone) Titration Pack B	Two titration packs per 365 days
Bysanti (milsaperidone) Titration Pack C	Two titration packs per 365 days
Desmoda (desmopressin acetate)	5 bottles (725 mL) per 30 days
Juxtapid (lomitapide) 2 mg capsule	1 capsule per day
Nereus (tradipitant)	60 tablets per 365 days
Ontralfy (tizanidine oral solution)	2,790 per 30 days
Opdivo Qvantig (nivolumab-hyaluronidase-nvhy) 300 mg nivolumab and 5,000 units hyaluronidase per 2.5 mL (120 mg/2,000 units per mL)	2 vials (5 mL) per 28 days
Pivya (pivmecillinam)	21 tablets per 7 days
Seysara (sarecycline)	1 tablet per day
Strensiq (asfotase alfa) injection 18 mg/0.45 mL, 28 mg/0.7 mL, 40 mg/mL	24 vials per 28 days
Strensiq (asfotase alfa) injection 80 mg/0.8 mL	48 vials per 28 days
Yartemlea (narsoplimab-wuug)	16 mL per 28 days
Yuvezzi (carbachol and brimonidine tartrate ophthalmic solution)	1 single dose vial per day
Yuviwel (navepegritide) 1.3 mg & 2.8 mg vials	4 kits (2.4 mL) per 28 days
Yuviwel (navepegritide) 5.5 mg vial	8 kits (8 mL) per 28 days
Zepbound (tirzepatide) Kwikpen	1 pen per 28 days
Zycubo (copper histidinate)	1 vial per day